BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Elapsed-time tests remain due if a cycle is delayed. The treatment team books, reviews and acts on results. Calendar timing alone does not authorise a dose.

Check	Before starting	Every 3-week cycle	Around week 12	Ongoing treatment	Treatment held / changed
Blood count / kidney function	FBC, renal profile and creatinine clearance	FBC and renal profile before each cycle; sooner for diarrhoea, dehydration or infection	Continue each-cycle tests	Interpret creatinine changes with hydration and true renal function; tucatinib can affect creatinine transport	Reassess renal function and capecitabine safety before restarting
Liver tests	ALT, AST and bilirubin	Every 3 weeks, or sooner if symptoms / abnormalities	Continue every 3 weeks	Report jaundice or dark urine promptly	Repeat abnormal tests and review tucatinib/capecitabine separately
DPD / capecitabine safety	Confirm pretreatment DPD result before first capecitabine; renal impairment can affect uracil phenotyping	Review early severe diarrhoea, mouth ulcers, infection or neurological symptoms immediately	No routine repeat DPD test simply because a cycle passes	Complete deficiency excludes capecitabine; partial deficiency requires an individual plan	Hold affected tablets and seek urgent advice; do not restart independently
Heart function	Cardiac history, ECG and baseline echo/MUGA under trastuzumab product information	Review breathlessness, swelling and chest pain at each visit	Repeat LVEF approximately 12-weekly under the NCCP/ individual cardiac plan	4–8-weekly if impairment; keep elapsed-time due dates when route changes	A qualifying LVEF decline requires hold and repeat within 3 weeks; post-treatment follow-up is planned separately
Diarrhoea / skin / mouth	Provide separate tablet-hold instructions and hydration plan	Review bowels, intake, hands/ feet and mouth before continuation	Symptoms can occur between visits and during capecitabine breaks	Tucatinib continues on the planned Days 15–21 capecitabine break unless specifically held	The medicines have different interruption and restart rules
Disease / CNS review	Baseline disease assessment and CNS plan when indicated	Clinical review; imaging not tied to tablet-break days	Response assessment commonly every 2–4 months, individualised	Earlier assessment for new symptoms; CNS schedule decided separately	Markers support clinical/ imaging assessment, not decisions alone

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00936 v1 · official regimen · ESC 2022 cardio-oncology guideline · cardiac surveillance and risk · ASCO · biomarkers in metastatic breast cancer · ESMO metastatic breast cancer guideline · response assessment · EMA Tukysa product information · retrieved 25 October 2026 · EMA Xeloda product information · retrieved 25 October 2026 · EMA Herceptin product information · retrieved 25 October 2026

THE WHOLE TREATMENT JOURNEY

Tucatinib + capecitabine + trastuzumab: treatment and checks

Advanced / metastatic · previously treated HER2-positive breast cancer

Tucatinib twice daily every day; capecitabine twice daily on Days 1–14 then off on Days 15–21; trastuzumab every 21 days by IV infusion or a locally approved under-the-skin injection. Continue while beneficial and tolerated, with clinician-approved changes.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Days 1–14	Days 15–21	Next cycle	Disease review
Cancer medicines NCCP 00936	Confirm prior HER2 therapies and DPD result	Tucatinib + capecitabine; trastuzumab Day 1 by agreed IV or SC formulation	Tucatinib continues; capecitabine break	Review before any restart / next infusion	Continue, modify or change treatment according to benefit / toxicity
Surgery METASTATIC	No routine curative surgery sequence from this regimen	Local procedures only if indicated	Coordinate interruptions with MDT	Review symptoms and functional needs	Site-specific / CNS MDT when needed
Radiotherapy METASTATIC	Assess need for symptom or brain-directed treatment	Any concurrent RT needs an explicit plan	Do not assume capecitabine can continue with RT	Coordinate holds / restart with radiation team	Imaging and local treatment depend on disease site
Blood tests NCCP 00936	FBC, renal, liver; DPD testing before capecitabine	Extra tests if symptoms / toxicity	Pre-cycle bloods and symptom review	FBC / renal per cycle; liver at least every 3 weeks	Check dehydration, hepatic toxicity and interactions
Heart checks NCCP 00936 · HERCEPTIN · ESC	Baseline cardiac assessment, ECG and echo/MUGA	Report chest pain or breathlessness	Symptoms need assessment between visits	Follow the recorded LVEF plan, approximately every 12 weeks	Risk / abnormalities may shorten interval
Cancer imaging METASTATIC · MARKERS	Baseline disease sites; brain assessment as indicated	No scan required simply because Day 14 is reached	Imaging schedule independent of tablet break	Usually review response about every 2–4 months; individualise	Use consistent modalities; earlier imaging if concern

What each check is for

1 Blood tests and treatment review

FBC, renal, liver; DPD testing before capecitabine → Extra tests if symptoms / toxicity → Pre-cycle bloods and symptom review → FBC / renal per cycle; liver at least every 3 weeks → Check dehydration, hepatic toxicity and interactions

Results may change whether a dose is given or held.

Ask / act: Confirm the booking and who reviews the result before treatment.

Sources: NCCP 00936

2 Heart tests

Baseline echo / LVEF → Report chest pain or breathlessness → Symptoms need assessment between visits → Repeat echo according to HER2 plan, commonly 12-weekly → Risk / abnormalities may shorten interval

ECG measures rhythm; echo measures pumping function. They are different checks.

Ask / act: Keep the planned appointments and report symptoms between them.

Sources: NCCP 00936 · ESC

3 Imaging and markers

Baseline disease sites; brain assessment as indicated → No scan required simply because Day 14 is reached → Imaging schedule independent of tablet break → Usually review response about every 2–4 months; individualise → Use consistent modalities; earlier imaging if concern

The purpose and frequency depend on early versus advanced disease.

Ask / act: Ask which test is next and what decision its result will inform.

Sources: METASTATIC · MARKERS

4 Who arranges and reviews each check?

At initiation and every handover

A recommended test can be missed if responsibility is unclear.

Ask / act: Ask the team to document the booking owner, result reviewer, due window and escalation route.

Sources: NCCP 00936

5 Trastuzumab formulation confirmed

Before first dose and any route change

IV and SC doses and missed-dose instructions differ.

Ask / act: Confirm the route and visit/observation plan with the team.

Sources: NCCP 00936 · HERCEPTIN

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-10-25 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.21